**جوستاف روسي انترناشيونال - مصر (EPE) ***

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** الهام عبدالفتاح محمد ابراهيم
**المحافظة :** القاهره **الحي / المدينة :** الشرابيه **العنوان :** +++++
**رقم البطاقة :** 27904090104002 **رقم التليفون :** 01114447058 **تاريخ تسجيل الإستمارة :** 18-10-2026
**العمر(تاريخ الميلاد) :** 09-04-1979 **المهنة :** حاصل علي بكالوريوس

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبطات الاروماتيز [14161] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: الهام عبدالفتاح محمد ابراهيم
National ID: 27904090104002

# تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |

23




National ID · Front
[[DOCTYPE:ID_FRONT]]




National ID - Back
[[DOCTYPE:ID_BACK]]

| | |
|---|---|
| **Name:** الهام عبد الفتاح محمد ابراهيم<br>**MR. :** 5474 | |

Detailed Medical Report

| | |
|---|---|
| **السيد** | |

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient was diagnosed with a luminal breast cancer and received neoadjuvant chemotherapy (NACTH) followed by right mastectomy (RT MRM) in 2021. Post-operative management included placement of a port and initiation of endocrine therapy with ARAMIDEX and ZOLADEX in May 2021, as well as ZOMETA for six months. ARAMIDEX was continued with supplemental therapy for three months, and a dose of ZOLADEX was scheduled to be administered on 21-02-2026.**

**Subsequent management included prescription of FEMARA and calcium supplementation (1 tablet daily, 90 days supply). The patient was also prescribed ONE ALPHA ZOLADEX 10.8 SC and ZOMETA 4 mg for a pause in therapy. Imaging studies ordered were a breast ultrasound and a chest X-ray, and a full laboratory panel was requested.**

**The patient's breast cancer is characterized as right-sided, clinical stage T3 N2, invasive ductal carcinoma, estrogen-receptor positive (8/8), progesterone-receptor positive (4/8), HER2-negative. The treatment plan recommended neoadjuvant chemotherapy with an anthracycline-based regimen and TAXOL, initiated on 03-03-2022, followed by right mastectomy (RT MRM) and postoperative radiotherapy (YPT2 YPN2). Ongoing hormonal therapy with ZOLADEX and ARAMIDEX has been continued since April 2022, with the patient remaining free of disease as of June 2026. The current plan is to continue ZOLADEX and ARAMIDEX as prescribed.**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | 19-09-2026 | **الوقت :** | |

**الطبيب المعالج :** ايه مدحت بدوي محمد

**يعتمد ,**

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية
السنت المصرية هلل صحة مصر

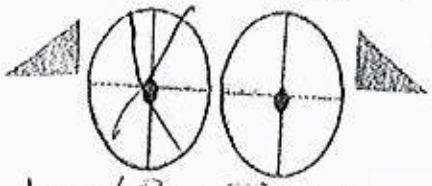
(٥٠٢)

# MDT FORM

| | |
|---|---|
| Name | الهام عبد الفتاح محمد |
| Age | ٤٢ سنة |
| MRN | |
| ID | ٠٤٠٤١١/١٩٠٤ |
| Co morbidity | LL DVT |

Rt MRM

٠١١٤٤٤٧٠٥٨
CV٩٠٤-٩٠١٠٤٠٠



brachial $CT_4N_2$
(4 AC, 4 Taxotere)

## Clinical Data

| | |
|---|---|
| Bx: 1st tumour site residual DCIS component 2nd small tumour site mixed IDC GII & mucoid ca GII | SM: |
| IHC: ER: PR: Her: Ki67: +ve metastatic deposits 7/15 LNs | Mets Workup: |
| free deep margin, nipple N M Areola | |
| COMMENT: partial pathological response $ypT_2ypN_2$ | |

## MDT OUTCOME & PLAN

9/4/2022 → - PORT
- Adj. Zoladex + A.I
(Consider extention)

| SIGNATURE | | | |
|---|---|---|---|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
| | | | |

الممسوحة ضوئيا بـ CamScanner
MDT Form
[[DOCTYPE:MDT_FORM]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / الهام عبد الفتاح محمد | Request No. | : 13008364 |
| Patient No. | : 27904090104002 | Request Date | : 03/03/2022 |
| Path No. | : 594/2022 | Verification Date | : 22/03/2022 |
| Sex | : Female | Financial Category | : اورام دار السلام - مبادره |
| Age | : 42 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data:
Right breast cancer. Post neoadjuvant chemotherapy.

Nature of the specimen:
Right MRM.

Gross Examination:
• Procedure: Right MRM specimen measured 26x17.5x6cm, covering skin piece measured 17x11cm. Nipple and areola were unremarkable grossly.
• Tumor extent: Two tumour bed sites were revealed:
1- First large tumour bed site: An ill-defined rubbery greyish white tumour bed site measured 6x4x2cm, 2cm away from deep margin.
2- second small tumour bed site: Another ill-defined rubbery greyish white tumour bed site measured 4x3.5x2cm.
• Lymph nodes number and size: 15 lymph nodes, up to 2x1.5cm.

Microscopic examination:
1- First large tumour bed site:
• Histological type: Residual DCIS components.
2- Second small tumour bed site:
• Histological type: Mixed invasive duct carcinoma grade 2 & mucoid carcinoma grade 2.
• Lymph node involvement: Positive metastatic deposits in (7/15) lymph nodes, with extracapsular infiltration.
• Resection Margins: Free deep margin.
• Free nipple and areola.
• Histologic response to therapy: Partial pathological response.
• Additional pathologic findings: No.

Verified by :
abdelrahman ibrahim



Room No. (4) - First Floor Building No. 87- Nile Corniche - Tor El Qadima - Cairo - Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | الأستاذه / الهام عبد الفتاح محمد : | Request No. | : 13008364 |
| Patient No. | : 27904090104002 | Request Date | : 03/03/2022 |
| Path No. | : 594/2022 | Verification Date | : 22/03/2022 |
| Sex | : Female | Financial Category | : اورام دار السلام - مبادره |
| Age | : 42 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

Diagnosis:
Right MRM:
1- First large tumour bed site: Residual DCIS components.
2- Second small tumour bed site: Mixed invasive duct carcinoma grade 2 & mucoid carcinoma grade 2.
- Positive metastatic deposits in (7/15) lymph nodes, with extracapsular infiltration.
- Free deep margin, nipple and areola.
- Partial pathological response.
- Stage: ypT2, ypN2

Prof. Dr. Amal Hareedy
Dr. Abdulrahman Ibrahim

Verified by :
abdelrahman ibrahim



Room No.(4) - First Floor Building No. 237 - Nile Corniche - Giza El Qadima Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name : Eiham Abdelfatah Date : 28/06/2021

## POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. **Chest:**

- Right breast retro-areolar and 12 o'clock enhancing irregular speculated mass lesion with overlying skin thickening measuring 3.8 X 3.5 cm.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 2.5 X 1.5 cm.
- Prominent right internal mammary lymph nodes measuring 1 X 0.5 cm.
- Prominent subcentimetric right sided supra-clavicular lymph node.
- Few bilateral scattered tiny pulmonary nodules ranging from 2-3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Heterogeneous texture of the thyroid gland.

2. **Abdomen and pelvis:**

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.

CT Scan
[[DOCTYPE:CT_SCAN]]

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Dilated elongated tortuous peri-uterine venous plexus.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows bilateral sacro-iliitis, no definite worrisome lytic or sclerotic lesions.
-

***OPINION:***

- ***Right breast locally advanced malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.***
- ***Few tiny bilateral scattered pulmonary nodules likely of benign nature for follow up.***
- ***No definite hepatic or osseous metastatic deposits.***

***Dr.Mohammed Mohsen, MSc.***

*Dr. M. Gomaa, M.D.*

CT Scan
[[DOCTYPE:CT_SCAN]]

Oncology Center
مركز الأورام
NASSER INSTITUTE
ONCOLOGY CENTER
NUCLEAR MEDICINE UNIT

وزارة الصحة
إدارة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital
مركز الأورام
وحدة الطب النووي

*Patient Name: Elham Abdel-Fatah Mohamed.* *Date: 7/11/2021*

# $^{99m}$Tc-$^{99m}$Tc-MDP BONE SCAN

**Desired information :** *skeletal surveying to detect multiple lesions.*

**Technique:** Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi $^{99m}$Tc-MDP:

## Report:

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No other definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Both kidneys are seen.

## *Conclusion:*

***The surveyed skeleton is free definite osseous metastasis.***

Thanks for reference

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

Alexandria El Nile - Agouza - Giza - Egypt - Cairo Tel: 33039164 - 33039356 Fax: 33033360

Bone Scan
[[DOCTYPE:BONE_SCAN]]

National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

# Pathology Report

Name: الهام عبدالفتاح محمد ابراهيم Age: 42years.
Clinical data: right breast mass
Specimen type: Tru cut biopsy.
Date (Received): 3/6/2021 Date(reported): 8/6/2021
Lab. Number: 1143/21 Hospital Number: 21/بدرية

## Gross Examination:

- Procedure: tru-cut biopsy
- Localization: N/A
- Tumor extent
  - Number: 4 cores
  - Size: up to 1cm.
  - Skin involvement: N/A
- Distance from margins: N/A
- Lymph nodes number and size: N/A
- Additional pathologic findings: N/A

Immunohistochemistry technique: Sections prepared from paraffin block for immune-staining using Beach Mark ULTRA system for ER, PR, HER2
ER clone (1E2)
PR clone (SP1).
HER-2 CLONE (PB5)

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Microscopic examination:

- Histological type: IDC
- Grade (differentiation - nuclear pleomorphism - mitosis): G (1+1+1=) II
- Angioinvasion: NO
- Ductal carcinoma in situ: no
- Tumor extent
  - Number:
  - Size:
  - Skin involvement:
- Lymph node involvement: N A
- Resection Margins:
  - Free /DCIS/ invasive carcinoma
- Additional pathologic findings:

*Diagnosis:*

**Rt breast mass, tru cut biopsy:**

- Invasive duct carcinoma type, GII.
- Biomarkers:
- ER: strong Intensity 80% nuclear reaction (Allred score 8/8)
- PR: moderate Intensity 20% nuclear reaction (Allred score 4/8)
- Her2: negative, faint Intensity >10%, with incomplete membrane reaction (score 1+)
- Ki67: 25% nuclear reaction

Prof. Gamal Abdel-Aty

Dr. Rana Fathy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]